

Manchester Royal Infirmary — Cardiology Department

Date: 28 Feb 2024

CLINIC LETTER

Patient: Margaret Thompson

DOB: 1954-08-15 (age 69)

NHS Number: 999 100 0001

Dear Dr Whitfield,

Thank you for referring Mrs Thompson. I reviewed her today in the heart failure clinic.

History: 6-week history of worsening exertional dyspnoea, NYHA class III. Orthopnoea on two pillows. Bilateral ankle oedema. No chest pain or palpitations. No recent admissions.

Examination: BP 138/82, HR 72 regular. JVP elevated 4 cm. Bibasal crackles. Pitting oedema to mid-shin bilaterally.

Investigations: Echocardiogram (today) confirms severe LV systolic dysfunction with LVEF 28% (down from 35% in 2022). Moderate mitral regurgitation. ECG: sinus rhythm, LBBB. Bloods: Na 138, K 4.6, urea 12.1, creatinine 168, eGFR 32. NT-proBNP 4200 pg/mL.

Allergies: Patient reports penicillin allergy (rash, 2018). Avoid beta-lactams.

Plan:

- Increase furosemide to 80 mg OD
- Add spironolactone 25 mg OD — monitor U&E in 2 weeks
- Consider sacubitril/valsartan in 4 weeks if eGFR stable
- Repeat eGFR and electrolytes in 2 weeks
- Referral to heart failure nurse specialist for education and titration
- Routine cardiology follow-up in 3 months

Yours sincerely,

Dr S. Mitchell, Consultant Cardiologist